BID DECISION BRIEF · SAUDI ARABIA · HEALTHCARE · 10 NOVEMBER 2026

Saudi National Dialysis PPP: *bid, partner or pass?*

Four packages, twelve qualified bidders and six years to make it pay. What a bid team should decide before the RFP lands.

11,500+	4	12	6 yrs
patients, minimum	regional packages	bidders qualified	DRFM + clinical term

BOTTOM LINE FOR BID TEAMS

Conditional bid. Integrated renal operators with Saudi delivery capacity should prepare for one or two packages and price upgrade capex to a six-year recovery. Hospital groups without renal scale should partner. Financial investors should not lead.

1 Deal snapshot
2 Procurement clock
3 Contract architecture
4 Capex-recovery maths
5 Competitive field
6 Risk allocation
7 Bid decision
8 RFP questions

VERIFIED official notice · REPORTED reputable press · FORMULA computed, inputs shown · ILLUSTRATIVE assumed inputs · ASSESSMENT author's judgement

1 Deal snapshot

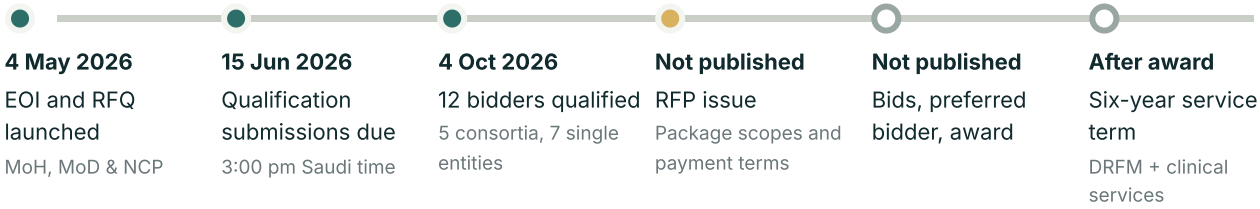
A national programme to buy chronic kidney disease and dialysis care from the private sector, packaged regionally and procured by the Ministry of Health, the Ministry of Defense and the National Center for Privatization (NCP).

Item	What is on the record	Basis
Procurers	Ministry of Health, Ministry of Defense, National Center for Privatization & PPP	VERIFIED
Model	PPP under a Design, Re-employ, Finance and Maintain (DRFM) framework, plus clinical services	VERIFIED
Term	Six-year contract	VERIFIED
Scale	At least 11,500 beneficiaries nationwide	VERIFIED
Packaging	Four clusters spanning several regions; final division set during bidding	VERIFIED
Private scope	Provide and upgrade facilities and equipment, run health IT, hire clinical and admin staff, run daily operations; in-centre and home haemodialysis, peritoneal dialysis, specialist outpatient care	VERIFIED
Field	12 qualified: 5 consortia and 7 single entities (4 Oct 2026)	VERIFIED
Not yet public	Package boundaries, payment basis, tariff, KPI and deduction regime, capex estimate, RFP and bid dates	VERIFIED gap

2 Procurement clock

FIGURE 1 VERIFIED dates from MoH notices

From qualification to RFP: the next gate has no published date



WHAT IT SHOWS
Each confirmed step since May 2026, and the steps still to come.

HOW TO READ IT
Teal = done. Gold = next gate. Grey = after award.

WHY IT MATTERS
Bid teams have a quiet window now to form consortia and model capex before the RFP fixes the rules.

AT A GLANCE

Time between gates so far: 42 days from launch to submissions; 111 days from submissions to qualification FORMULA . At a similar pace, teams that start pricing only when the RFP arrives will be short of time.

3 Contract architecture

The private partner does not just run clinics. It upgrades buildings, buys equipment, runs IT and employs clinicians, then hands it all back after six years. That combination decides who can win and how it must be financed.

FIGURE 2 ASSESSMENT structure as described in the MoH notice

Who does what, per package

GOVERNMENT

MoH, MoD and NCP define packages, buy the service, monitor KPIs and take the facilities back at handback.

PRIVATE PARTNER (PER PACKAGE)

Design upgrades · **R**e-employ existing sites · **F**inance capex · **M**aintain assets · deliver **clinical services** for six years.

SUPPLY CHAIN

Dialysis machines, water treatment, consumables and drugs, health IT, staffing pipeline. Often the consortium's second member.

WHAT IT SHOWS
The three blocks a winning bid must cover.

HOW TO READ IT
Any block a bidder cannot deliver in-house needs a partner or a subcontract.

WHY IT MATTERS
Five of the twelve qualified bidders are already consortia: operators are pairing with suppliers and local groups.

What analysis can answer before the RFP

MEASURED DIRECTLY

Minimum patient numbers, package count, term, scope, qualified field.

PROXIES & ASSUMPTIONS

Sessions per patient, upgrade capex per site, cost of capital, staffing cost per station.

JUDGEMENT

Which packages suit you, how hard to price volume risk, who to partner with.

THE STRUCTURAL TENSION

A **DRFM** contract asks the private side to **finance** capex, but a **six-year** term is shorter than a typical project-finance loan for healthcare assets. Expect corporate or equipment finance, not non-recourse project debt, unless the RFP adds an extension or a capex payment ASSESSMENT.

Source: Saudi MoH notice of 4 May 2026 (scope and DRFM framework)

4 Capex-recovery maths

The six-year term is the single most important number in this deal. It fixes how much every dialysis session must carry to repay upgrade and equipment spending.

METHOD 4.1

Capital recovery charge per session

FORMULA

CALCULATION

$$\textbf{Charge per session} = \text{Capex} \times r \div (1 - (1 + r)^{-n}) \div \text{Sessions per year}$$

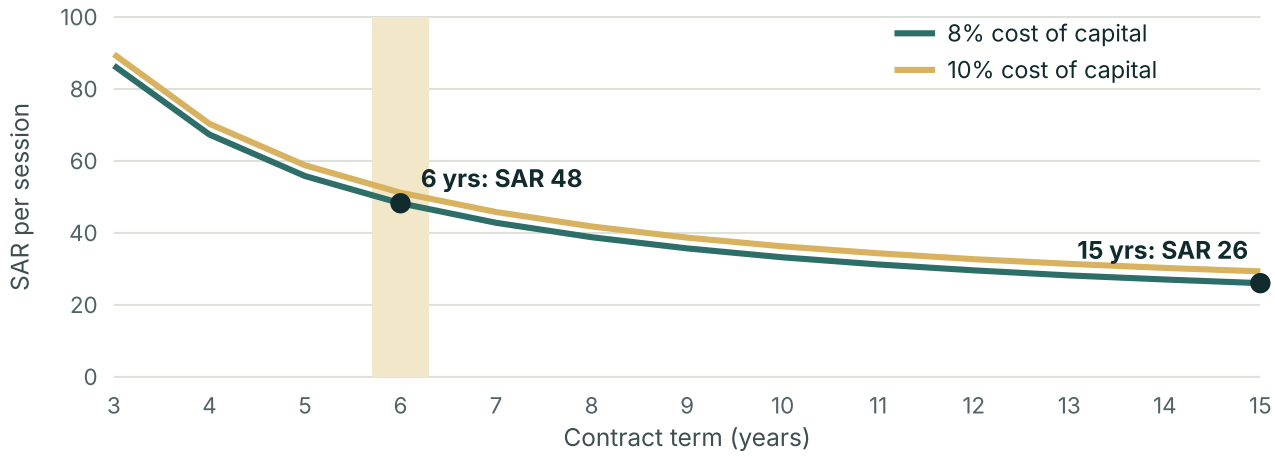
where r is the cost of capital and n the term in years. The first part is the standard annuity that repays capex with a return; dividing by annual sessions gives the amount each treatment must carry.

WORKED EXAMPLE · ILLUSTRATIVE INPUTS

One package at the even split: 2,875 patients FORMULA × 156 sessions a year (three a week, assumed all in-centre) ILLUSTRATIVE = 448,500 sessions. Capex SAR 100m ILLUSTRATIVE at 8%: annual charge SAR 21.6m → **SAR 48 per session over 6 years**, against **SAR 26 over 15 years**. The six-year term almost doubles the capex load per session (×1.85).

FIGURE 3 ILLUSTRATIVE per SAR 100m of capex, 448,500 sessions a year

Capex charge per session falls fast as the term lengthens



WHAT IT SHOWS

SAR each session must carry to repay SAR 100m of capex, by term and cost of capital.

HOW TO READ IT

Scale linearly: SAR 50m of capex halves every figure; fewer sessions raise it.

WHY IT MATTERS

At six years the curve is still steep. One year more or less moves price materially; ask for an extension option.

Term	Annual charge, SAR m (8%)	SAR per session (8%)	SAR per session (10%)
6 years	21.6	**48**	51
10 years	14.9	**33**	36
15 years	11.7	**26**	29

Inputs labelled illustrative are assumptions, not RFP data. Home and peritoneal dialysis reduce in-centre sessions.

5 Competitive field

#	Qualified bidder (4 Oct 2026)	Form
1	National Medical Care Co. + NephroTech	Consortium
2	DiMed Saudi + SMC + Saedan	Consortium
3	Gulf Medical Co. + B. Braun	Consortium
4	NephroPlus + Faisaliah Medical Systems	Consortium
5	Weigao + Anfas	Consortium
6	Burjeel Arabia Healthcare	Single entity
7	DaVita Healthcare	Single entity
8	Diaverum Advanced Medical Services	Single entity
9	NephroCare Saudi Medical Co.	Single entity
10	US Renal Care	Single entity
11	Dr. Sulaiman Al Habib Medical Group	Single entity
12	Al Mana General Hospitals	Single entity

THE ARITHMETIC

12 ÷ 4 = 3

qualified bidders per package, if all bid everything and packages are equal FORMULA . Rules on bids per bidder are not yet public.

WHAT THE FIELD SAYS

Five consortia pair operators with suppliers or local groups. Several global renal specialists and large Saudi hospital groups qualified in their own right ASSESSMENT.

IMPLICATION

Competition will be on operating cost and clinical delivery, not on financing. A bidder without a cost edge per session should target the package where its existing footprint is strongest.

6 Risk allocation: one risk, three views

Risk	Government	Lender	Sponsor / operator
Patient volume below plan	Wants a minimum service guarantee met	Needs a revenue floor before lending	Prices a volume band or walks
Upgrade capex overrun	Fixed scope, fixed price	Sized on 6-year cash; little room	Carries the overrun; equity first loss
Clinical quality and KPIs	Deductions tied to outcomes	Deductions cap matters for DSCR	Operator skill is the moat
Staffing and Saudisation	Local-content targets	Cost line, rarely a lender issue	Wage and hiring risk for six years
Consumables and drug prices	Seeks a stable tariff	Indexation protects margin	Pass-through or hedge via supplier
Handback after year 6	Takes facilities and equipment back	Debt must be repaid by then	Residual value of equipment at risk

ASSESSMENT Typical positions for a service PPP of this type; the RFP will set the actual allocation.

Source: Saudi MoH notice of 4 Oct 2026 (qualified list)

7 Bid decision

FIGURE 4 ASSESSMENT

Should your firm bid? Four gates

1. Do you deliver dialysis clinically at scale, or have that operator in your consortium?	No →	**Partner or pass.** Join an operator as supplier, local partner or minority investor.
2. Can you staff and run clinics in Saudi Arabia for six years, meeting local-content targets?	No →	**Add a Saudi partner** before the RFP, or limit the bid to one package.
3. Does your price recover upgrade capex within six years at your cost of capital (Figure 3)?	No →	**Bid capex-light**: re-use sites as found, lease equipment, or make an extension a condition.
4. Can your balance sheet carry the investment without long-tenor project debt?	No →	**Line up equipment or corporate finance** with tenor of six years or less now.
All four answered yes	→	**Bid** for the one or two packages where your footprint and referral base are strongest.

Verdict by bidder type

Bidder type	Verdict	Why
Integrated renal operator	BID 1–2 PACKAGES	Owns the cost curve per session and the clinical model
Hospital group without renal scale	PARTNER	Brings sites and referrals; needs a renal operator
Equipment or consumables maker	CONSORTIUM MEMBER	Supply margin plus equipment-finance role
Financial investor	MINORITY ONLY	Short term, operator-driven returns, little leverage
Bank or lessor	LEND ≤ 6 YEARS	Equipment or corporate facilities matched to the term

Bankability scorecard

Criterion	Reason	Score
Offtaker strength	MoH / MoD are the counterparties	●●●●● 5/5
Revenue certainty	Payment basis not yet published	●●○○○ 2/5
Tenor fit	Six years is short for capex recovery	●●○○○ 2/5
Construction risk	Re-purposed sites, not greenfield	●●●●○ 4/5
Operating risk	Clinical delivery for six years	●●○○○ 2/5
Competition	~3 qualified bidders per package	●●●○○ 3/5
Disclosure	RFP and package scopes pending	●●○○○ 2/5
Overall	Strong counterparty, thin disclosure, short tenor	**2.9 / 5 · Conditional**

Author's assessment from public information; to be re-scored when the RFP is issued

8 Twelve questions to put to the procurers

Each answer moves price or bankability. Bid teams should table them at the first clarification round.

1. Payment basis: per session, per patient, availability, or a mix?
2. Minimum volume or referral commitment per package?
3. Indexation formula for tariff, wages and consumables?
4. KPI regime: which clinical outcomes trigger deductions, and is there a cap?
5. Who owns equipment at handback, and at what condition?
6. Any extension option beyond six years?
7. Transfer of existing MoH / MoD staff: mandatory, optional, cost?
8. Limit on packages per bidder, and can one bid win two?
9. Integration with national health IT systems and data-hosting rules?
10. Termination compensation for each cause, including government default?
11. Treatment of home haemodialysis and peritoneal dialysis volumes in payment?
12. Bid bond, performance bond and parent-guarantee sizes?

Signals to watch

Signal	What it would change
RFP issue date	Starts the pricing clock; reveals package boundaries and payment basis
Payment basis	Per-session pay moves volume risk to bidders; availability pay moves it to government
Extension option	Each extra year lowers the capex charge per session (Figure 3)
Bids-per-bidder rule	Decides whether large operators can sweep packages

Sources

Source	Used for
Saudi Ministry of Health, notice 4 May 2026 · moh.gov.sa/en/ministry/mediacenter/ads/pages/ads-2026-05-04-001.aspx	Procurers, DRFM framework, six-year term, four packages, 11,500 minimum, scope, 15 June deadline
Saudi Ministry of Health, news 4 Oct 2026 · moh.gov.sa/en/Ministry/MediaCenter/News/Pages/news-2026-10-04-001.aspx	Twelve qualified bidders and their names
Zawya · zawya.com (Saudi Arabia invites bidders for CKD care and national dialysis PPP)	Independent confirmation of packages, patients and term

ABOUT THIS BRIEF

Prepared by Ramesh Radhakrishnaraja, FCA, with AI assistance; every fact is checked against the cited sources as at 10 November 2026. Illustrative figures use assumed inputs and are labelled. This is an analytical view of a public procurement, not legal or investment advice, and not a recommendation to any specific bidder.

Soli Deo Gloria